

CHESAPEAKE NEUROLOGY ASSOCIATES

4801 Ritchie Hwy, Suite 310 | Brooklyn Park, MD 21225

Phone: (410) 555-0193 Fax: (410) 555-0197

Provider NPI: 1234567890

OUTPATIENT NEUROLOGY REFERRAL COVER SHEET

TO: Johns Hopkins Hospital -- Dept. of Neurology

Fax: (410) 614-2828

Attn: General Neurology / Memory & Movement (see notes)

FROM: Dr. Preethi Nambiar, MD

Chesapeake Neurology Associates

DATE FAXED: March 14, 2025

TOTAL PAGES INCLUDING COVER: 11

(Note: some pages may be out of order due to EHR export -- sorry)

PATIENT: GARRISON, WALTER D.

DOB: 09/22/1948 AGE: 76

MRN (CNA): 00274-1193

INS: Medicare Part B / Supplemental (Aetna) ID# H4471-00293B

REASON FOR REFERRAL:

Pt referred for evaluation of progressive gait difficulty, recurrent falls, and possible cognitive issues. Unclear etiology -- query vascular vs. degenerative. Rule out NPH vs. other. ?Parkinsonism. Family requesting second opinion.

***** Please route to appropriate subspecialty as needed *****

URGENCY: Routine (non-emergent)**Preferred Appt Window:** Within 4-6 weeks if possible**Interpreter needed:** No**Mobility limitations:** YES -- uses walker, difficulty with transfers

Attached documents (may be incomplete/partial scan):

- Demographics / Insurance page
- Referring provider note (03/10/2025)
- Prior clinic notes x3 (various dates)

- Labs (CBC, CMP, TSH, B12, RPR, UA -- various dates)
- MRI Brain 02/18/2025 -- report only, images on CD mailed separately
- CT Head 11/03/2024 (report)
- Medication list (current as of 03/10/2025)
- Discharge summary (St. Agnes Hospital, 01/08/2025)
- LP report 02/21/2025

Questions: call Dr. Nambiar's nurse line (410) 555-0193 x204

***** CONFIDENTIAL MEDICAL INFORMATION -- FAX TO AUTHORIZED RECIPIENT ONLY *****

CHESAPEAKE NEUROLOGY ASSOCIATES -- PATIENT DEMOGRAPHICS

** EHR EXPORT -- PRINTED 03/14/2025 **

PATIENT INFORMATION

Last Name: GARRISON
First Name: WALTER
Middle Initial: D.
Date of Birth: 09/22/1948
Age: 76 years
Sex: Male
Race/Ethnicity: White / Non-Hispanic (patient reported)
SSN: ***-**-6612 (on file)
Marital Status: Married
Language: English (primary)

CONTACT INFORMATION

Address: 7720 Quarterstaff Road
Ellicott City, MD 21043
Home Phone: (443) 555-0284
Cell: (443) 555-0391 (wife -- primary contact)
Email: [not on file]

EMERGENCY CONTACT

Name: Diane Garrison (wife)
Relationship: Spouse
Phone: (443) 555-0391

INSURANCE / BILLING

Primary: Medicare Part B
Medicare #: 1EG4-TE5-MK72 Eff: 01/01/2014
Secondary: Aetna Medicare Supplement (Plan G)
Aetna Member ID: H4471-00293B
Group #: 00293
Referral Auth #: AUTH-2025-03-88124 (valid 90 days from 03/10/2025)

REFERRING PROVIDER

Provider: Preethi Nambiar, MD -- Neurology
Practice: Chesapeake Neurology Associates
NPI: 1234567890
Fax: (410) 555-0197

PRIMARY CARE PROVIDER

Provider: Thomas J. Calloway, MD

Pt: GARRISON, Walter D. DOB: 09/22/1948

CNA MRN: 00274-1193

Practice: Ellicott City Family Medicine

Phone: (410) 555-0442

Fax: (410) 555-0448

NOTE: Pt states Dr Calloway "not really involved" -- wife manages medications

CHESAPEAKE NEUROLOGY ASSOCIATES

Outpatient Clinic Visit -- Progress Note

Date of Service: 03/10/2025 Electronically signed: 03/11/2025 09:14

Provider: Preethi Nambiar, MD (Neurology)

Visit Type: Established patient -- 99214

REASON FOR VISIT:

76M presenting for follow-up re: progressive gait instability and falls. Wife present and very concerned. Patient initially seen here approximately 8 months ago for same concerns, referred to PT at that time. Has not improved. Had hospitalization in January (see d/c summary attached -- St. Agnes) after fall with head strike. No LOC per wife. Now requesting Hopkins referral.

HPI:

Walter is a 76-year-old retired postal worker with a history of HTN, hyperlipidemia,

T2DM (diet controlled), and remote L4-L5 lumbar discectomy (2011). Over the past 18-24 months wife notes he has become progressively slower in his movements,

has developed a shuffling quality to his gait, and has had multiple falls -- approximately 6-8 falls over the past year by her count. Falls occur both with turning and also when transitioning from seated to standing. He sometimes freezes

at doorways per wife. He has been seen by his PCP (Dr. Calloway) who attributed this initially to "arthritis and deconditioning." Patient himself minimizes symptoms.

Wife reports he has also been more forgetful -- misplaces items, repeats questions.

She is unsure if this is new vs. longstanding. No clear history of stroke. No incontinence currently -- though she notes he had some urgency issues earlier this

year (resolved per patient). No syncope. No hallucinations reported by wife today

(though see prior note -- this is possibly inconsistent).

ROS otherwise negative in detail per attached template.

PMHx:

Hypertension (dx ~2003)

Hyperlipidemia

Type 2 Diabetes Mellitus -- diet controlled (per patient, per PCP)

Lumbar disc disease -- L4-L5 discectomy 2011 (Johns Hopkins)

Mild chronic low back pain (residual)

BPH -- on tamsulosin

Basal cell carcinoma -- R cheek, excised 2019 (clear margins)

GERD

Osteoarthritis bilateral knees

Anxiety NOS -- on low dose lorazepam (wife disagrees this is indicated)

Hard of hearing -- bilateral, wears aids (compliance poor per wife)

SurgHx: L4-L5 discectomy 2011, BCC excision 2019, cataract surgery R eye 2021

FamHx:

Father: deceased, Parkinson disease (dx age ~72, died age 81)
Mother: deceased, Alzheimer disease
Sister: alive, essential tremor

SocHx:

Retired letter carrier (USPS), 30 years. Married x51 years. Lives w/ wife in 2-story home -- primarily uses first floor now. Non-smoker (quit 1989). Remote tobacco hx (20 pack-yr). EtOH: occasional beer, < 3/week per wife. No illicit drug use.

MEDICATIONS (see full list attached -- brief summary):

Lisinopril 10mg QD, Amlodipine 5mg QD, Atorvastatin 40mg QHS,
Omeprazole 20mg QD, Tamsulosin 0.4mg QHS, Lorazepam 0.5mg QHS PRN,
Aspirin 81mg QD, Metformin 500mg BID (started per PCP -- pt unsure if still taking), MiraLax PRN, Vitamin D 1000IU QD, Fish oil 1g QD

ALLERGIES: Sulfa (rash), Codeine (nausea)

PHYSICAL EXAM:

Vitals: BP 148/88 (R arm, sitting) HR 72 RR 16 SpO2 97% RA
Temp 98.2F Wt 181 lbs Ht 5'9" BMI 26.7

General: Elderly male, appears stated age, pleasant, mildly hard of hearing. Moves slowly. Wife answers most questions.

HEENT: NC/AT, hearing aids bilateral. PERRL 3mm bilat. EOMI -- I did note slight difficulty with upward gaze, pt somewhat uncooperative with exam.

Neck: Supple, no adenopathy.

CV: RRR, no murmurs. Peripheral pulses intact.

Resp: CTA bilaterally.

Abdomen: Soft, NT/ND.

Extremities: No edema. No cyanosis.

NEUROLOGIC:

MS: Alert. Oriented x2 (person, place; not date). MoCA administered: scored 19/30 (lost pts: orientation, attention, delayed recall x3, visuospatial). Limited by hearing.

CN: See above (gaze concern). Facial symmetry intact. Facial masking noted. Voice soft, low volume (hypophonia).

Motor: Tone increased bilaterally -- more pronounced R > L upper extremity. Rigidity present (cogwheel R wrist). Strength grossly 4+/5 throughout proximal UE, 4/5 proximal LE. No focal weakness.

Tremor: Minimal resting tremor noted R hand -- pill-rolling quality. Wife confirms this is intermittent -- not always present.

Reflexes: 2+ throughout, symmetric. Plantar responses flexor bilat.

Sensation: Decreased light touch distal LE bilat (likely length-dependent).

Cerebellar: No limb ataxia on FNF (limited cooperation).

Gait: Observed from hallway. Stooped posture. Stride length markedly reduced. Arm swing reduced bilaterally (R>L). Turns en bloc. Difficulty initiating -- festination noted on one occasion. Uses walker but still unsteady. Pull test performed -- retropulsion, multiple steps to recover.

(Note: unable to perform TUG formally today -- pt refused standing unaided)

ASSESSMENT:

76M with progressive gait disorder, postural instability, and falls. Exam notable for bradykinesia, rigidity (R>L), hypophonia, facial masking, postural instability, and en bloc turning. Mild cognitive impairment on screening.

Family history significant for PD (father). Presentation raises concern for parkinsonian syndrome -- etiology unclear at this time. Differential includes:

1. Parkinson disease (idiopathic)
2. Atypical Parkinsonism (MSA, PSP, or DLB given cognitive/ocular findings)
3. Vascular parkinsonism (significant WM changes on MRI -- see report)
4. Normal pressure hydrocephalus (gait, cognition -- but tremor/rigidity less typical; LP done -- see report; incontinence not clearly present now)
5. Drug-induced (lorazepam, metformin -- reviewed; no antipsychotics on board)

PLAN:

1. Referral to Johns Hopkins Neurology for further evaluation and subspecialty guidance re: movement disorder vs. other etiology.
2. Continue current medications -- discussed risk of lorazepam in elderly; wife reluctant to discontinue ("it's the only thing that helps him sleep")
3. Fall precautions reinforced. Walker use mandatory.
4. PT/OT referral renewal submitted.
5. f/u with me in 6-8 weeks or sooner if falls increase.

Electronically signed: Preethi Nambiar MD 03/11/2025 09:14 AM

CHESAPEAKE NEUROLOGY ASSOCIATES

Outpatient Clinic Visit -- Progress Note (PRIOR NOTE)

Date of Service: 07/09/2024 Provider: Preethi Nambiar, MD

Electronically signed: 07/10/2024 10:52

REASON FOR VISIT:

[AUTO-POPULATED FROM TEMPLATE -- NEW PATIENT VISIT 99203]

New patient evaluation. Referred by Dr. Calloway (PCP) for gait difficulty and falls x ~6 months.

HPI:

Mr. Garrison is a 75-year-old male presenting with complaints of unsteady gait and 2-3 falls in the past 6 months. Wife accompanies. Patient walked in today without assistive device (wife reports he usually refuses to use walker). Denies tremor. Denies memory problems. Wife notes some slowness in activities of daily living, difficulty buttoning shirt, handwriting changed. Falls occur without clear prodrome -- no presyncope, no LOC reported. PCP obtained CBC, CMP, TSH, B12 (see outside labs -- all reportedly normal per patient; records not in hand).

PMHx / FamHx / SocHx: [See HPI above; full history not re-documented this visit per office policy -- refer to intake form on file]

Medications at this visit (per patient/wife):

Lisinopril, Atorvastatin, Aspirin, Tamsulosin, Omeprazole, Vitamins
(Lorazepam NOT mentioned at this visit)

PHYSICAL EXAM:

Vitals: BP 152/90 HR 74 Wt 184 lbs

General: Well-appearing male, pleasant. Moves slowly but steadily today in exam room. No apparent distress.

Neurologic:

MS: Alert, oriented x3. Conversational. No formal cognitive testing today.
(Note: wife feels this was "a good day" for him)

Motor: Tone mildly increased R arm. No clear cogwheel.
Possible very slight resting tremor R hand -- not sustained.

Gait: Mildly reduced stride. Arm swing somewhat diminished R. Turns okay.
(Wife: "he is much worse at home")

Reflexes: 2+ sym. Plantars flexor.
No dysmetria noted.

ASSESSMENT:

Gait instability, etiology unclear. Exam today not dramatically abnormal -- may have orthopedic component given back surgery history. R/O degenerative vs. peripheral neuropathy. Will obtain imaging and labs. PT referral placed.
[No specific neurologic diagnosis assigned this visit]

PLAN:

1. MRI brain and lumbar spine without contrast (ordered -- see imaging below)
2. Repeat labs: CBC, CMP, TSH, B12, folate, RPR, HbA1c
3. Outpatient PT for gait training, strengthening
4. Return to clinic in 3 months or sooner if worsening

Signed: P. Nambiar MD 07/10/2024

CHESAPEAKE NEUROLOGY ASSOCIATES

Outpatient Clinic Visit -- Progress Note (PRIOR NOTE)

Date of Service: 10/22/2024 Provider: Preethi Nambiar, MD

Signed: 10/22/2024 17:31 [SIGNED SAME DAY -- END OF DAY BATCH]

REASON FOR VISIT:

Follow up. Pt seen July 2024 -- gait and falls. Wife called 2 weeks ago requesting sooner appointment due to increased falls (was scheduled for Jan).

INTERVAL HPI:

76M (copy-forward: 75M -- please disregard; pt birthday 09/22 -- now 76) with worsening gait instability. PT has been going 2x/week x 8 weeks -- minimal improvement per wife. 4 additional falls since July -- one with knee contusion (Rx at urgent care, no fracture). One fall while trying to get up in the night. Wife now sleeping in same room because she is worried. Patient continues to minimize. Wife has noticed what she describes as visual hallucinations -- twice in past month, patient reported seeing "animals in the yard that weren't there." Patient denies this when asked directly. She also notes fluctuating alertness -- some days much more confused than others. Has had urinary urgency -- using pads. MRI lumbar spine from July: multilevel DDD, spinal stenosis L3-4, L4-5 -- per ortho spine (Dr. Watkins) who saw him in September: "not a surgical candidate, gait likely not primarily from spine." (Ortho note not included here -- sorry)

MRI Brain (from outside -- St. Agnes 08/2024, report only -- no images):

[NOTE: This is a DIFFERENT scan from the JHH MRI ordered 02/2025]

Report states: "Moderate to confluent periventricular and subcortical white matter

T2/FLAIR hyperintensities consistent with chronic small vessel ischemic change. Mild generalized volume loss. No acute infarct. Ventricles mildly prominent -- likely involutional. No enhancing lesion. DWI negative."

[Radiologist: W. Chen, MD -- St. Agnes Radiology -- 08/12/2024]

PHYSICAL EXAM:

Vitals: BP 144/86 HR 70 SpO2 96% Wt 179 lbs (down 5 lbs from July)

General: Slower today than prior visit. Wife assisting with coat, answers first.

Neurologic:

MS: Alert but mildly confused today -- wife says "medium day." MoCA 21/30
(different from later note -- please note variability)

Facial masking more apparent. Hypophonia. Blink rate seems reduced.

Motor: Rigidity bilateral (R>L). Bradykinesia on rapid alternating movements.
Resting tremor R hand clearly present today -- pill-rolling.

Gait: Uses walker (finally). Marked shuffling. Freezing at threshold (observed).

En bloc turning. Significant retropulsion on pull test.

Wife: "this is actually a good day for walking."

Ocular: Downward saccades appear slow on exam today. Upward as well.

[Querying vertical gaze palsy -- not definitively present, needs

formal neuro-ophthalmology assessment]

ASSESSMENT/PLAN:

Progressive parkinsonism -- concern now for atypical features (cognitive fluctuations, possible VH, possible vertical gaze palsy, falls early in course). Cannot exclude NPH given ventriculomegaly, gait, and incontinence -- though tremor/rigidity unusual for classic NPH. Will order repeat MRI w/ NPH protocol. Referral to Hopkins placed (pending insurance auth). Continue fall precautions. Discussed discontinuing lorazepam -- family declined at this time.

Do NOT start dopaminergic therapy until specialist evaluation

P. Nambiar MD 10/22/2024 [DRAFT SIGNED -- ADDENDUM MAY FOLLOW]

ELLICOTT CITY FAMILY MEDICINE

9301 Baltimore National Pike, Suite 100 | Ellicott City, MD 21042
Thomas J. Calloway, MD | NPI 9876543210

OUTPATIENT PROGRESS NOTE (PRIOR NOTE -- FROM PCP)

Date of Service: 12/04/2024 Signed: 12/04/2024 18:14
[EHR System: AthenaHealth -- Auto-populated fields marked with *]

CHIEF COMPLAINT: *Follow up - multiple chronic conditions*

SUBJECTIVE:

Mr. Garrison in for routine f/u. Wife present. Concerns: gait continues to worsen. Has neurology follow-up scheduled. Had a fall last week -- no injury. BP running high at home (systolics 150-160s). Diabetes: diet-controlled, he admits "not great" with diet. Wife mentions he seems more confused at times -- she is frustrated that nobody has given a clear diagnosis. He is sleeping poorly.

Appetite fair. Weight stable per patient (wife says he has lost weight but she does not have a scale).

ROS: Positive for falls, gait instability, sleep disturbance, confusion

ROS: Negative for chest pain, SOB, dysuria, fever (template)

OBJECTIVE:

Vitals: BP 162/94 HR 68 RR 16 Temp 97.8 Wt 181 lbs BMI 26.7

General: Elderly male. Moving slowly. Hearing aids. Pleasant.

Neuro: A+O x2-3. No focal deficits noted. Gait -- shuffling, walks with cane

[NOTE: neurology notes walker -- patient uses cane to PCP office per wife as walker "doesn't fit in the car"]. Mild hand tremor.

CV: RRR. No M/R/G. LE no edema.

Resp: CTA. Skin: healing right cheek scar (BCC per hx). Abdomen: soft, NT.

ASSESSMENT / PLAN:

1. Hypertension -- poorly controlled. Increase amlodipine to 10mg QD.
Consider adding second agent if not improved at f/u.
2. T2DM -- A1c 6.8% (October labs). Continue Metformin 500mg BID.
Dietary counseling reinforced.
3. Gait instability / falls -- deferred to neurology. Will send records.
Patient and wife frustrated with pace of workup.
4. BPH -- stable on tamsulosin.
5. GERD -- stable on omeprazole.
6. Anxiety / sleep -- lorazepam 0.5mg QHS. Aware of risks in elderly. Wife insists this is helping. Documented discussion re: Beers Criteria. Rx continued
with plan to reassess.
7. Osteoarthritis knees -- patient declines ortho referral at this time.
Recommends Tylenol PRN. (Avoid NSAIDs given renal function.)
8. Hyperlipidemia -- LDL 81 on atorvastatin (October). Stable.
9. Hard of hearing -- scheduled for repeat audiogram.
RTC 6-8 weeks or sooner if needed.

Signed: Thomas J. Calloway MD 12/04/2024

Copy to: Nambiar MD (Neurology -- CNA)

LABORATORY RESULTS -- COMPILED*(Multiple sources -- dates vary -- see individual entries)*

CHESAPEAKE NEUROLOGY ASSOCIATES -- Ordered 07/15/2024 Resulted 07/16/2024**CBC WITH DIFFERENTIAL:**

WBC	7.2 K/uL	[4.5-11.0]	NORMAL
RBC	4.51 M/uL	[4.5-5.9]	NORMAL
Hemoglobin	13.8 g/dL	[13.5-17.5]	NORMAL
Hematocrit	41.2 %	[41.0-53.0]	NORMAL
MCV	88 fL	[80-100]	NORMAL
Platelets	241 K/uL	[150-400]	NORMAL
Neutrophils	64%	Lymphs 28%	Monos 6% Eos 2%

CMP (COMPREHENSIVE METABOLIC PANEL):

Sodium	139 mEq/L	[136-145]	NORMAL
Potassium	4.1 mEq/L	[3.5-5.1]	NORMAL
Chloride	102 mEq/L	[98-107]	NORMAL
CO2	26 mEq/L	[22-29]	NORMAL
BUN	22 mg/dL	[7-25]	NORMAL
Creatinine	1.1 mg/dL	[0.7-1.3]	NORMAL
eGFR	68 mL/min/1.73m2		mildly reduced (CKD stage G2)
Glucose	112 mg/dL	[70-99]	HIGH *
Calcium	9.3 mg/dL	[8.6-10.3]	NORMAL
AST	28 U/L	[10-40]	NORMAL
ALT	31 U/L	[7-56]	NORMAL
Alk Phos	74 U/L	[44-147]	NORMAL
Total Bili	0.8 mg/dL	[0.2-1.2]	NORMAL

THYROID:

TSH	2.14 uIU/mL	[0.4-4.0]	NORMAL
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VITAMIN B12 / FOLATE:

B12	384 pg/mL	[200-900]	NORMAL
Folate	>20 ng/mL		NORMAL

RPR (SYPHILIS SEROLOGY):

RPR	NON-REACTIVE	NORMAL
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HbA1c (10/14/2024 -- Ellicott City FM lab):

HbA1c	6.8%	[<5.7 normal; 5.7-6.4 prediabetes; >6.5 DM]
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ELEVATED -- consistent with diabetes

URINALYSIS (01/06/2025 -- collected at St. Agnes ED):

Color: Yellow Appearance: Cloudy
Specific Gravity: 1.018 pH: 6.0
Glucose: Trace Protein: Trace Blood: Negative
Nitrite: POSITIVE * Leukocyte Esterase: POSITIVE *
WBC >50/hpf Bacteria: Many
[UTI -- treated w/ Bactrim x5d per d/c summary; no longer on antibiotics]

URINE CULTURE (01/07/2025 -- St. Agnes):

E. coli > 100,000 CFU/mL -- susceptible to TMP/SMX, nitrofurantoin

RADIOLOGY -- CT HEAD WITHOUT CONTRAST

Date: 11/03/2024 Facility: St. Agnes Hospital Radiology

Ordering Provider: Nambiar MD (order transmitted via fax 10/30/2024)

CLINICAL INDICATION: Gait instability, falls, rule out intracranial pathology, rule out NPH.

TECHNIQUE: CT of the brain without intravenous contrast.

FINDINGS:

There is no evidence of acute intracranial hemorrhage, large territory infarct, or mass lesion. Periventricular and deep white matter hypodensities are present, compatible with chronic small vessel ischemic changes.

The ventricular system is mildly enlarged, out of proportion to cortical atrophy. The sylvian fissures are mildly widened. Calvarium intact.

No subdural collection. Skull base unremarkable. Paranasal sinuses clear.

IMPRESSION:

1. Chronic periventricular white matter changes -- small vessel ischemic disease.

2. Mildly enlarged ventricular system. Clinical correlation recommended to exclude normal pressure hydrocephalus.

3. No acute intracranial process.

Signed: R. Okonkwo, MD (Radiology) 11/03/2024 16:22

RADIOLOGY -- MRI BRAIN WITH AND WITHOUT CONTRAST

Date: 02/18/2025 Facility: Johns Hopkins Outpatient Radiology, Green Spring

NOTE: IMAGES ON CD -- MAILED TO REFERRING PROVIDER 02/20/2025

CLINICAL INDICATION: Parkinsonism, rule out atypical parkinsonian syndrome, NPH, other structural etiology. Gait instability, cognitive decline.

TECHNIQUE: Multiplanar, multisequence MRI brain with and without IV gadolinium. DWI, ADC, FLAIR, T1, T2, GRE, SWI sequences obtained.

FINDINGS:

BRAIN PARENCHYMA: Moderate to severe confluent periventricular and subcortical white matter T2/FLAIR signal abnormality, markedly greater than prior CT 11/2024. Findings most consistent with advanced small vessel ischemic change. Mild cortical atrophy, somewhat disproportionate to stated age. The midbrain appears mildly atrophic -- the midbrain/pons area ratio is at the lower limit of normal. No "hummingbird sign" definitively identified on sagittal T1, though

borderline. No substantia nigra signal abnormality appreciated on SWI.

VENTRICLES: Moderately enlarged lateral, third, and fourth ventricles out of proportion to sulcal atrophy. Evans' index 0.33 (>0.3 threshold met).

Transependymal flow noted. No aqueductal stenosis.

DWI/ADC: No restricted diffusion to suggest acute infarct.

POST-CONTRAST: No abnormal enhancement.

SWI: No microhemorrhages. No iron deposition pattern suggestive of MSA.

IMPRESSION:

1. Advanced periventricular and subcortical white matter disease -- vascular etiology favored. Cannot exclude contribution to parkinsonism.

2. Ventriculomegaly satisfying radiographic criteria for NPH (Evans' index 0.33) with transependymal flow. Clinical and CSF correlation needed.

3. Borderline midbrain atrophy -- PSP not excluded; clinical correlation and subspecialty evaluation recommended.

4. No definitive substantia nigra or basal ganglia signal change.

Signed: M. Patel, MD (Neuroradiology) 02/18/2025 14:07

CURRENT MEDICATION LIST

Chesapeake Neurology Associates -- As of 03/10/2025

***** MEDICATION RECONCILIATION NOT FULLY VERIFIED -- SOME MEDS PER PATIENT/WIFE REPORT *****

MEDICATION	DOSE	ROUTE	FREQ	PRESCRIBER
Lisinopril	10 mg	PO	QD	Calloway
Amlodipine	10 mg	PO	QD	Calloway
(NOTE: increased from 5mg Dec 2024 -- some pharmacy records still show 5mg)				
Atorvastatin	40 mg	PO	QHS	Calloway
Aspirin	81 mg	PO	QD	Calloway
Omeprazole	20 mg	PO	QD	Calloway
Tamsulosin	0.4 mg	PO	QHS	Calloway
Metformin	500 mg	PO	BID	Calloway
(patient unsure if still taking -- wife says "sometimes forgets both doses")				
Lorazepam	0.5 mg	PO	QHS PRN	Calloway
*** BEERS CRITERIA -- benzodiazepine in elderly; documented discussion ***				
(wife reports using nightly, not PRN; patient taking ~4-5x/week per wife)				
Polyethylene glycol (MiraLax)	17 g	PO	PRN	Calloway
Vitamin D3	1000 IU	PO	QD	OTC
Fish oil (omega-3)	1000 mg	PO	QD	OTC

DISCONTINUED / UNCLEAR STATUS:

Ibuprofen 200mg (OTC) -- wife says he was taking occasionally for knee pain until advised to stop by PCP (renal function). Date stopped unclear.

Melatonin 3mg QHS -- wife reports he tried this briefly but stopped.

Calcium + D supplement -- found in home, unclear if taking.

ALLERGIES:

Sulfonamides -- RASH (reported; type/severity not documented)

Codeine -- NAUSEA

NKDA (per PCP note Dec 2024 -- CONFLICT with above -- please verify)

PHARMACIST COMMENT (CNA in-house review, 03/10/2025):

Lorazepam -- falls risk, cognitive effects in elderly; recommend deprescribing.

Tamsulosin -- orthostatic hypotension risk; relevant given fall history.

Metformin -- renally dose appropriate at current eGFR (68). Continue.

Aspirin 81 -- continue given vascular risk factor burden.

No known CYP450 interactions identified.

ST. AGNES HOSPITAL

900 Caton Avenue | Baltimore, MD 21229
Main: (410) 368-6000

DISCHARGE SUMMARY

[THIS IS A FAX COPY -- ORIGINAL ON FILE AT ST. AGNES HIM]

Patient: GARRISON, WALTER D.
DOB: 09/22/1948 MRN (St. Agnes): 00883201
Admission Date: 01/06/2025
Discharge Date: 01/08/2025
LOS: 2 days
Attending: Sunita Kapoor, MD (Hospitalist)
Service: Internal Medicine

ADMISSION DIAGNOSIS: Fall with head injury (unwitnessed); evaluation for intracranial injury; UTI incidentally found.

DISCHARGE DIAGNOSES:

1. Fall with head contusion -- no intracranial hemorrhage on CT
 2. Urinary tract infection (E. coli) -- treated
 3. Hypertension -- continued home medications
 4. Gait disorder, etiology under outpatient workup (per neurology)
 5. Acute on chronic confusion -- attributed to UTI; improved at discharge
-

HOSPITAL COURSE:

Mr. Garrison is a 76-year-old male with history of hypertension, diabetes, and known gait instability under neurologic workup who presented after an unwitnessed fall at home on the evening of 01/05/2025. Wife found him on the kitchen floor. He denied LOC but was confused at presentation. No focal neurologic deficits on ED exam. CT Head without contrast obtained -- see report (no hemorrhage, no fracture, WM changes as before). Admitted for observation and for confusion workup. UA showed UTI -- E. coli, started on TMP-SMX (confirmed susceptible on culture, no sulfa allergy documented in their system -- please NOTE: per CNA records pt has sulfa allergy; this was flagged on discharge, pt tolerated without reaction during admission however). Confusion improved with UTI treatment. Neurology consulted in-house: Dr. A. Prasad (neurology resident, St. Agnes) examined pt 01/07/2025: noted bradykinesia, mild rigidity, and gait abnormality. Recommended outpatient movement disorder evaluation. No changes to neurologic meds.

[CONSULT NOTE NOT INCLUDED -- WAS NOT AVAILABLE AT TIME OF FAX]

PT evaluated -- recommended walker and home PT. Patient worked with PT x1. Discharged home with wife in stable condition. BP 138/82 on discharge.

DISCHARGE MEDICATIONS (CHANGES FROM ADMISSION):

NEW: TMP/SMX DS PO BID x5 days (to complete course) -- note allergy conflict
CHANGED: Amlodipine increased to 10mg QD (on recommendation of attending)

CONTINUED: all other home medications unchanged

FOLLOW UP:

PCP (Dr. Calloway) within 1-2 weeks

Neurology (Dr. Nambiar) -- appointment already scheduled

Home PT arranged

Signed: Sunita Kapoor, MD 01/08/2025 11:44

Copy to: Dr. Calloway (PCP), Dr. Nambiar (Neurology)

CHESAPEAKE NEUROLOGY ASSOCIATES**PROCEDURE NOTE -- LUMBAR PUNCTURE (DIAGNOSTIC / NPH TAP TEST)**

Date of Procedure: 02/21/2025

Provider: Preethi Nambiar, MD (Neurology)

Location: CNA Procedure Suite, Suite 315

INDICATION:

76M with progressive gait disorder, cognitive decline, ventriculomegaly on MRI (Evans' index 0.33), and prior urinary urgency. Rule out normal pressure hydrocephalus. Large-volume LP performed as diagnostic tap test. Pre- and post-LP gait testing performed.

CONSENT: Written consent obtained from patient and wife.

POSITION: Left lateral decubent.

SITE: L3-L4 interspace (confirmed by palpation; ultrasound not used).

TECHNIQUE: Fluoroscopy-free. Standard 20g Sprotte needle. Opening pressure measured with manometer.

OPENING PRESSURE: 18 cm H2O (reference: 6-20 cm H2O -- NORMAL)*[Note: Pt was cooperative but tense; may affect OP slightly]*

CSF REMOVED: 30 mL (large volume tap)

CSF STUDIES:

Color: Colorless

Clarity: Clear

RBC: 2 /uL (ref: 0-5)

WBC: 1 /uL (ref: 0-5)

Differential: 100% lymphocytes

Glucose: 62 mg/dL (serum glucose 106 at time of LP -- ratio nl)

Protein: 48 mg/dL (ref: 15-45) BORDERLINE HIGH *

Xanthochromia: Negative

VDRL (CSF): Non-reactive

Oligoclonal bands: NOT SENT (not ordered per plan)

14-3-3 protein: NOT SENT

Alpha-synuclein: NOT SENT

Cytology: NEGATIVE for malignant cells (routine screen)

POST-LP GAIT ASSESSMENT:

Gait testing performed pre-LP and 1 hour post-LP (30 mL removed).

Pre-LP TUG: 31.4 seconds (highly abnormal; ref: < 12 sec)

Post-LP TUG (1hr): 28.1 seconds

Pre-LP gait velocity: 0.42 m/s

Post-LP gait velocity: 0.47 m/s

Wife reports: "he seemed a little better -- steadier -- after the procedure"

INTERPRETATION: Minimal objective improvement post-LP (< 20% change in TUG).

This does NOT strongly support NPH tap-test response. However, assessment was limited by patient fatigue, testing within procedure suite (not optimal walking conditions), and anxiety. Subjective improvement reported by wife.

[Result equivocal -- further specialist evaluation warranted]

COMPLICATIONS: None. Pt tolerated procedure well. No post-LP headache at 1hr. Instructed to call if headache develops. Wife given written instructions.

Signed: Preethi Nambiar MD 02/21/2025 15:03

END OF FAXED DOCUMENT -- LAST PAGE

TRANSMISSION VERIFICATION

Sent by: Sandra Lopes, MA (Medical Assistant -- CNA)
Date/Time: 03/14/2025 07:42 AM
Destination: JHH Neurology Referral Line: (410) 614-2828
Pages sent: 11 (including cover)
Status: CONFIRMED (transmission receipt attached to original in chart)

ADDITIONAL NOTES FROM REFERRING OFFICE:

- MRI images from 02/18/2025 (Hopkins Green Spring) have been mailed on CD to Hopkins neurology. CD sent 03/13/2025 via USPS priority mail.
 - Neuropsychological testing has NOT been performed. If Hopkins deems appropriate, family is amenable.
 - DaTscan was discussed with patient and wife; they have NOT yet agreed to this. Insurance preauth not yet submitted.
 - St. Agnes inpatient neurology consult note (01/07/2025) was NOT obtained; facility could not locate record to send. Family was not given a copy.
 - Ortho spine note (Dr. Watkins, Sep 2024) not included -- was not released by their office in time for this referral.
 - Please contact Sandra at (410) 555-0193 x200 if any pages missing/illegible.
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